

Texas WIC Health and Human Services Commission

Effective July 1, 2025

Policy No. BF:01.0

Breastfeeding Promotion and Support Standards

Purpose

To provide optimal nutrition and encourage WIC participants to breastfeed their infants. To provide support to breastfeeding participants so that each may reach their personal breastfeeding goal.

Authority

7 CFR Part 246.11

Policy

The WIC staff and the clinic environment must promote and support breastfeeding, and encourage participants to exclusively breastfeed for 6 months, unless medically contraindicated, and continue to breastfeed with the addition of complementary foods for the first 2 years of life and thereafter as long as mutually desired.

Definitions

Breastfeeding – the practice of feeding a mother’s breastmilk to her infant(s) on the average of at least once a day.

WIC-Designated Breastfeeding Expert (DBE) – An individual who is an expert with special experience or training in helping breastfeeding mothers and who provides breastfeeding expertise and care for more complex breastfeeding problems when WIC staff face situations outside their scope of practice. Individuals with this designation can be WIC staff including Breastfeeding Coordinators, International Board Certified Lactation Consultants (IBCLCs), dietitians, nutritionists and nurses. The IBCLC is the preferred WIC DBE.

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Procedures

- I. The local agency (LA) must provide a breastfeeding-supportive clinic environment ([Policy GA:25.0](#)).
 - A. All staff must promote the importance of breastfeeding and use positive language when discussing breastfeeding.
 - B. Breastfeeding must be visibly represented through use of wall art or other visuals in the clinic.
 1. Clinics must store infant formula, bottles or pacifiers where applicants and participants cannot see them except while being used as a teaching aid.
 2. All materials distributed or visible to participants must:
 - i. Be free of formula product names and/or logos. This also includes office supplies such as cups, pens and notepads.
 - ii. Be free of language that undermines a mother's confidence in her ability to breastfeed.
 - iii. Show breastfeeding in a positive light.
 - iv. Be at an appropriate reading level for WIC participants.
 - v. Be culturally appropriate.
 - vi. Be accurate and up to date.
 - vii. Be state agency (SA) developed, or if LA developed, must be reviewed and approved by an LA IBCLC or Registered Dietitian (RD). If an LA IBCLC or RD is not available, LA must submit the material to the SA for approval.
 - C. Staff must encourage breastfeeding anywhere in the clinic, including the waiting room, and provide a private, comfortable space for those wishing to breastfeed or express milk in private.
- II. The LA must offer client-centered breastfeeding education to all pregnant participants.

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- A. Staff must offer client-centered breastfeeding education to all pregnant participants and encourage breastfeeding unless contraindicated for medical reasons.
 - 1. Staff must assess a pregnant participant's knowledge, attitude and concerns related to breastfeeding and identify the factors that may affect her ability to meet her breastfeeding goals. This must occur as part of the certification process.
 - i. Prenatal breastfeeding education can be provided by the peer counselor (PC), certifying authority (CA), WIC certification specialist (WCS), DBE or IBCLC.
 - 2. All pregnant women must be offered the opportunity to receive one on one support from a PC.
 - 3. Staff must offer all pregnant participants the following items at their first prenatal visit:
 - i. Prenatal Breastfeeding Education Bag
 - a. If preassembled bags are not available, staff must offer at minimum:
 - 1. Breastfeeding Guide: How to Get Off to a Great Start (stock no. 13-220/A),
 - 2. Man's Guide to Breastfeeding Support (stock no. 13-53/A)
 - 3. From One Generation to Another: A Grandmother's Guide to Breastfeeding (stock no. 13-06-11288/A).
 - ii. WIC Food Packages for Moms and Infants brochure (stock no. 13-06-13124-T and 13-06-13124A-T)
 - a. When reviewing this material, staff must include counseling focused on the importance of routine milk removal for establishing and maintaining milk supply.
 - b. Staff must tell participants that WIC can tailor food packages based on their infant feeding needs.

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- iii. Client Breastfeeding Resource List described in Procedure III.B.2.
 - B. Breastfeeding education and recommendations must be in alignment with current evidence-based recommendations.
- III. The LA must provide breastfeeding support.
- A. Every breastfeeding WIC participant must be offered breastfeeding support.
 - 1. PCs should be utilized first, when possible, for BF counseling that falls within their scope of practice. ([Policy BF:03.0](#))
 - 2. IBCLCs, DBEs, CAs and WCs must be utilized when PCs are not available.
 - B. Every WIC clinic must maintain and have available the following breastfeeding referral documents.
 - 1. Staff Breastfeeding Referral Protocol – a flowchart that provides staff the appropriate referral process to use when they have a breastfeeding situation or question that is beyond their own scope of practice or level of expertise.
 - i. All staff must have access to the protocol to refer to when needed.
 - ii. This protocol must be reviewed and updated annually and only include no cost services.
 - 2. Client Breastfeeding Resource List – a resource handout that provides a list of Texas WIC local and statewide breastfeeding support services as well as additional no-cost community breastfeeding resources, if available. This handout must:
 - i. include only no cost evidence-based resources.
 - ii. include the WIC non-discrimination statement ([Policy CR:02.0](#)).
 - iii. be updated each fiscal year.

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- iv. be offered to every pregnant and breastfeeding participant at each certification appointment and as needed.
- C. Hands-off/hands-on breastfeeding assistance:
 - 1. Hands-off approach - WIC staff must first try to assist a breastfeeding mother/baby dyad in a hands-off type approach by using verbal coaching and models or other appropriate tools.
 - 2. Hands-on approach - When there is a need to assist a mother through touch, WIC staff must always wash their hands or use hand sanitizer. WIC staff must also wear gloves while assisting.
 - i. WIC staff must ask permission and obtain verbal or written consent prior to touching.
 - ii. If the mother does not consent to be touched, WIC staff must use a hands-off approach.
- D. When a mother who is breastfeeding asks for formula or an increase in formula, staff must provide counseling and support.
 - 1. During counseling, staff should:
 - i. identify mother's personal breastfeeding goal(s) and discuss any concerns and barriers. Maintain a collaborative, client-centered approach.
 - ii. support breastfeeding to the maximum extent possible with minimal supplementation of infant formula.
 - iii. document the interaction in the management information system (MIS).
 - 2. Staff must make food package decisions after counseling has occurred and in alignment with [Policy FD:10.0](#).
- IV. All local WIC staff must receive BF training according to [Policy TR:03.0](#).
- V. All funds used to promote and support breastfeeding must be an allowable expense according to [Policy AC:34.0](#).

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